

# Medical Device Reporting for Manufacturers

---

## Guidance for Industry and Food and Drug Administration Staff

*This guidance represents the Food and Drug Administration's (FDA's) current thinking on this topic. It does not create or confer any rights for or on any person and does not operate to bind FDA or the public. You can use an alternative approach if the approach satisfies the requirements of the applicable statutes and regulations. If you want to discuss an alternative approach, contact the FDA staff responsible for implementing this guidance. If you cannot identify the appropriate FDA staff, call the appropriate number listed on the title page of this guidance.*

### 1. Introduction

#### 1.1 What is the purpose of this guidance document?

This guidance document describes and explains the Food and Drug Administration's (FDA, we, us) current regulation that addresses reporting and recordkeeping requirements applicable to manufacturers of medical devices for device-related adverse events and certain malfunctions.<sup>1</sup> These requirements are contained in our Medical Device Reporting (MDR) regulation at Title 21, Code of Federal Regulations (CFR), [Part 803](#), as authorized by section 519 of the Federal Food, Drug, and Cosmetic Act (FD&C Act). References to FDA regulations and Federal Register documents, as well as cross-references within this guidance document, are hyperlinked for your convenience.

FDA's guidance documents, including this guidance, do not establish legally enforceable responsibilities. Instead, guidances describe the Agency's current thinking on a topic and should be viewed only as recommendations, unless specific regulatory or statutory requirements are cited. The use of the word *should* in guidance documents means that something is suggested or recommended, but not required.

---

<sup>1</sup> FDA published a final rule entitled "Medical Device Reporting: Electronic Submission Requirements" (79 Fed. Reg. 8832, February 14, 2014, <http://www.gpo.gov/fdsys/pkg/FR-2014-02-14/html/2014-03279.htm>). FDA has modified this guidance based on that final rulemaking to ensure that the recommendations in this guidance are consistent with the final rule.

## 1.2 What is the purpose of the MDR regulation?

The MDR regulation provides a mechanism that allows us (the FDA), as well as you (the device manufacturer), to identify and monitor adverse events (deaths and serious injuries) and certain malfunctions involving your medical devices. The goal is to detect and correct problems in a timely manner. The requirements of the MDR regulation are enforced under the authority of the FD&C Act. The enforcement mechanisms include seizure, injunction, civil money penalties, and criminal prosecution.

The MDR regulation also includes adverse event reporting and recordkeeping requirements for medical device user facilities (e.g., a hospital, ambulatory surgical facility, nursing home, outpatient diagnostic facility, outpatient treatment facility), importers of medical devices, and medical device distributors.

## 1.3 What are the basic requirements of the MDR regulation that apply to me?

Manufacturers, including foreign manufacturers, of legally marketed medical devices in the United States are required to:

- Submit to us reports of [MDR reportable events](#)<sup>2</sup> involving their medical devices [21 CFR [803.10\(c\)](#) and [803.50](#)];
- Develop, maintain, and implement written procedures for the identification and evaluation of all medical device events (e.g., malfunctions, serious injuries and deaths) to determine whether the event is an MDR reportable event [21 CFR [803.17](#)] (for related information, see sections [2.10](#) and [3.1](#) of this guidance); and
- Establish and maintain complete files for all complaints concerning medical device events [21 CFR [803.18](#)] (for related information, see section [3.2](#) of this guidance).

## 1.4 What are “MDR reportable events”?

For manufacturers, “MDR reportable events” are events that manufacturers become aware of that reasonably suggest that one of their marketed devices may have caused or contributed to a death or serious injury, or has malfunctioned and the malfunction of the device or a similar device that they market would be likely to cause or contribute to a

---

<sup>2</sup> The MDR regulation published on December 11, 1995 ([60 FR 63578](#)) also included requirements for manufacturers to submit Annual Certification using FDA Form 3381 and Baseline Reports using FDA Form 3417. The Annual Certification requirement was revoked by the Food and Drug Administration Modernization Act of 1997 and removed from the regulation as published on January 26, 2000 ([65 FR 4112](#)). The Baseline Reporting requirement was removed from the regulation effective October 27, 2008 ([73 FR 33692](#) and [73 FR 53686](#)).

death or serious injury if the malfunction were to recur [see 21 CFR [803.3](#)] (see Section 2 ) of this guidance document for a description of “[serious injury](#)” and “[malfunction](#)” and information for determining when a device malfunction must be reported to us as an “MDR reportable event”).

### **1.5 Is this guidance also intended for user facilities, importers and distributors?**

No, this guidance document is intended for only medical device manufacturers. Certain requirements applicable to user facilities, importers, and distributors are summarized in Appendix A to facilitate better understanding of the requirements applicable to manufacturers because manufacturers may receive information about MDR reportable events from these entities. Note that this guidance does address manufacturing activities of user facilities, importers, and distributors that also manufacture devices.

### **1.6 Where can I obtain help if I have questions about this guidance document or the MDR regulation?**

If you have any questions or concerns regarding this guidance document or the MDR regulation, contact:

MDR Policy Branch  
Division of Postmarket Surveillance  
Office of Surveillance and Biometrics  
Center for Devices and Radiological Health  
10903 New Hampshire Avenue  
Building 66, Room 3217  
Silver Spring, MD 20993-0002

Telephone: 301-796-6670

E-mail: [MDRPolicy@fda.hhs.gov](mailto:MDRPolicy@fda.hhs.gov)

## 2. Manufacturer Reporting Requirements

### 2.1 What are the reporting requirements that apply to me as a medical device manufacturer?

As a manufacturer of medical devices, you are required to submit reports to us of a reportable death, serious injury, or device malfunction [21 CFR 803.10(c)]. A reportable death, serious injury, or malfunction is based on information a manufacturer receives or otherwise becomes aware of, from any source, which reasonably suggests that one of its marketed devices:

- May have caused or contributed to a death or serious injury [21 CFR 803.3 and 803.50] (see also sections 2.5 and 2.13 of this guidance); or
- Malfunctioned and the malfunction of the device or a similar device marketed by the manufacturer would be likely to cause or contribute to a death or serious injury if the malfunction were to recur [21 CFR 803.3 and 803.50] (see also section 2.14 of this guidance).

MDR reportable events generally must be submitted to us within 30 calendar days after the day you become aware of the event [21 CFR 803.10 and 803.50] and are referred to in this guidance document as “30-day reports” or “initial reports.” Under some circumstances, a medical device report is required to be submitted within 5 work days after the day you become aware of the need to submit such a report [21 CFR 803.10(c)(2) and 803.53].<sup>3</sup> Reports required to be submitted within 5 work days are referred to in this guidance document as “5-day reports” or “five-day reports.” A “work day” is Monday through Friday, except Federal holidays [21 CFR 803.3]. All types of required reports are explained further in this guidance document (see Sections 2.18, 2.19, and 2.20 of this guidance).

As stated in the Federal Register Notice , [76 FR 12743](#), Title II/Section 227 of the Food and Drug Administration Amendments Act of 2007 (FDAAA) (Public Law 110-85), amended Section 519(a) of the FD&C Act, 21 U.S.C. 360i(a) in relation to certain malfunction reporting requirements. The FR notice explains that FDAAA changed malfunction reporting requirements to quarterly summary reporting for all class I devices and those class II devices that are not permanently implantable, life supporting or life sustaining. However, pending further FDA notice, these devices currently remain subject to individual reporting requirements under 21 CFR Part 803 in order to protect the public

---

<sup>3</sup> You must submit a 5-day report to us no later than 5 work days after the day that you become aware that: (a) an MDR reportable event necessitates remedial action to prevent an unreasonable risk of substantial harm to the public health; or (b) we have made a written request for the submission of a 5-day report [21 CFR 803.53].

health. Consequently, malfunction reports for such devices are currently required to be reported under 21 CFR Part 803, as was required pre-FDAAA.

The notice also states that FDA intends to publish in the Federal Register a list of the types of devices that continue to be subject to 30-day reporting under 21 CFR part 803 in order to protect the public health under section 519(a)(1)(B)(i)(III) of the FD&C Act, 21 U.S.C. 360i(a)(1)(B)(i)(III). In addition, FDA intends to, by rulemaking, establish malfunction reporting criteria for devices subject to quarterly summary reporting per section 519(a)(1)(B)(ii) of the FD&C Act, 21 U.S.C. 360i(a)(1)(B)(ii).[\[80 FR 50010\]](#) In the interim, all device manufacturers and importers must continue to report malfunctions as individual reports in full compliance with 21 CFR part 803. FDA plans to modify this guidance based on any future notice to modify the types of devices subject to malfunction reporting under section 519(a)(1)(B)(i)(III) of the FD&C Act, 21 U.S.C. 360i(a)(1)(B)(i)(III), and any final rulemaking to establish criteria for malfunction reporting under section 519(a)(1)(B)(ii) of the FD&C Act, 21 U.S.C. 360i(a)(1)(B)(ii), to ensure, as needed, that the recommendations in this guidance are consistent with the final rule.

## **2.2 Who is considered to be a manufacturer subject to the reporting requirements of the MDR regulation?**

A “manufacturer” is any person who manufactures, prepares, propagates, compounds, assembles, or processes a device by chemical, physical, biological, or other procedure [21 CFR [803.3](#)]. The term includes any person who:

- Repackages or otherwise changes the container, wrapper, or labeling of a device in furtherance of the distribution of the device from the original place of manufacture;
- Initiates specifications for devices manufactured by a second party for subsequent distribution by the person initiating the specifications; or
- Manufactures components or accessories that are medical devices and that are (1) ready to be used and are intended to be commercially distributed and intended to be used as is, or (2) processed by a licensed practitioner or other qualified person to meet the needs of a particular patient.

All manufacturers of legally marketed medical devices in the US, including foreign manufacturers who export devices to the US, are subject to the MDR regulation and must submit required reports. US manufacturers of medical devices that are not cleared or approved in the US, but are exported to foreign locations, are also subject to the MDR regulation. Failure to submit required reports is a prohibited act [section 301(q) of the FD&C Act, 21 U.S.C. 331(q)] (however, see Section [4.11.1](#) of this guidance for the circumstances under which FDA generally does not intend to enforce MDR reporting requirements for those devices exported under sections 801(e) or 802 of the FD&C Act).

Any person who reprocesses a single use device for reuse in human beings becomes the manufacturer of the device and is subject to all the requirements applicable to the original manufacturer, including the requirements of the MDR regulation [[69 FR 7490](#)].

## **2.3 When do I “become aware” that an MDR reportable event has occurred?**

As a manufacturer, you are considered to have “become aware” of an event whenever [21 CFR [803.3](#)]:

- Any of your employees becomes aware of information that reasonably suggests that an event is required to be reported in a 30-day report or in a 5-day report that we have requested from you; or
- Any of your employees with management or supervisory responsibilities over persons with regulatory, scientific or technical responsibilities (including consultants or contractors) or whose duties relate to the collection and reporting of adverse events, becomes aware from any information (including any trend analysis) that an MDR reportable event(s) necessitates remedial action to prevent an unreasonable risk of substantial harm to the public health. In this case, you must submit a report no later than 5 work days after the day that you become aware.

Your MDR procedures should identify the roles and responsibilities of contractors or consultants that work for you or on your behalf to review and process complaints.

Please note: The Quality System (QS) regulation requires you to have a formally designated unit to receive, review and evaluate complaints [21 CFR [820.198\(a\)](#)]. Manufacturers who are subject to the QS regulation should ask all of their employees to forward device-related complaints to that unit in a timely manner (see also sections [2.10](#), [2.11](#), [2.12](#) and [3.1](#) of this guidance). Complaints should be evaluated in order to determine if they represent a reportable event in accordance with 21 CFR 803.3(o).

## **2.4 What is “information that reasonably suggests” that an MDR reportable event has occurred?**

This means any information, including professional, scientific, or medical facts, observations, or opinions that would cause you to come to a reasonable conclusion that a device has caused or may have caused or contributed to an MDR reportable event [21 CFR [803.20\(c\)](#)]. For guidance on when an adverse event does not have to be reported to us, see section [2.16](#) of this guidance.

## **2.5 What is meant by “caused or contributed” to a death or serious injury?**

This means that a death or serious injury was or may have been attributed to a medical device or that a medical device was or may have been a factor in a death or serious injury, including events occurring as a result of [21 CFR [803.3](#)]:

1. Failure;
2. Malfunction;
3. Improper or inadequate design;
4. Manufacture;
5. Labeling; or
6. User error.

## **2.6 What is device “user error” and why do you want to know about events involving user error?**

We consider a device “user error” (or “use error”) to mean a device-related error or mistake made by the person using the device. The error could be the sole cause of an MDR reportable event, or merely a contributing factor. Such errors often reflect problems with device labeling, the user interface, or other aspects of device design. Thus, FDA believes that these events should be reported in the same way as other adverse events which are caused or contributed to by the device. This is especially important for devices used in non-health care facility settings. If you determine that an event is solely the result of user error with no other performance issue, and there has been no device-related death or serious injury, you are not required to submit an MDR report, but you should retain the supporting information in your complaint files.

## **2.7 What form must I use to submit an MDR reportable event?**

Your submission must use an electronic equivalent of Form 3500A which is approved by FDA [[21 CFR 803.11](#) and [803.14](#)]. We have identified mandatory adverse event and reportable malfunction event reports as types of documents that must be submitted electronically.<sup>4</sup> You must submit reports of individual adverse events to FDA in an electronic format in accordance with [[21 CFR 803.12\(a\)](#)] and [[21 CFR 803.20](#)], unless granted an exemption under [[21 CFR 803.19](#)]. (See also section [2.31](#) of this guidance for information about the electronic submission of MDR events).

## **2.8 What information must be included in my report?**

Your report must contain all the information required by 21 CFR [803.52](#) that is known, or reasonably known to you. Information we consider reasonably known to you includes any information [21 CFR [803.50\(b\)](#)]:

- That you can obtain by contacting a user facility, importer, or other initial reporter;

---

<sup>4</sup> The CDRH May 8, 2008 notification is posted on the Part 11 Docket at [FDA-1992-S-0039-0054](#).

- That is in your possession; or
- That you can obtain by analysis, testing or other evaluation of the device.

Instructions for completing the Form 3500A, and information regarding codes to be used for specific sections of the form may be accessed via the following links:

Instructions for completing Form 3500A:

<http://www.fda.gov/downloads/Safety/MedWatch/HowToReport/DownloadForms/UCM387002.pdf>.

Information on the current Event Problem Code Hierarchy:

<http://www.fda.gov/MedicalDevices/DeviceRegulationandGuidance/PostmarketRequirements/ReportingAdverseEvents/EventProblemCodes/ucm134751.htm>.

Instructions for electronic reporting: [electronic Medical Device Reporting \(eMDR\) - Home Page](#)

## **2.9 Can I include multiple devices in the same report?**

Although a user facility may submit one report for an adverse event that involves multiple suspect devices, a manufacturer should submit a separate report for each device involved in the MDR reportable event.<sup>5</sup> For example, if you receive a report from a user facility which indicates that more than one of your devices may have contributed to one MDR reportable event, and you cannot determine which device actually caused or contributed to the event or which device malfunctioned, you should submit a separate report for each device potentially involved in the event.

## **2.10 What do you consider to be a device-related complaint and do I have to evaluate the complaint?**

For the purposes of this guidance document, we recommend that you use the definition of a complaint found in the QS regulation, which defines a complaint as:

[A]ny written, electronic, or oral communication that alleges deficiencies related to the identity, quality, durability, reliability, safety, effectiveness, or performance of a device after it is released for distribution [21 CFR [820.3\(b\)](#)].

You are required to review and evaluate all device-related complaints to determine whether the complaint represents an MDR reportable event [21 CFR [803.18\(e\)](#) and [820.198](#)].

## **2.11 Where will device-related complaints come from?**

---

<sup>5</sup> See <http://www.fda.gov/downloads/Safety/MedWatch/HowToReport/DownloadForms/UCM387002.pdf>.



You may receive complaint information from many different sources, including telephone calls or other verbal communication, FAX transmissions, written correspondence, e-mail notification, sales representative reports, service representative reports, scientific articles (literature), internal analyses, and legal documents. You may receive reports directly from user facilities, importers, or voluntary reporters. We may also send you a communication, such as a copy of a user facility or voluntary report.

FDA believes that manufacturers have a responsibility to inform all employees, including marketing, sales, engineering, manufacturing, regulatory, legal, installation, and service personnel, to immediately forward adverse event information to the appropriate person appointed by those entities to submit MDR reports. Thus, FDA generally considers that a manufacturer becomes aware of an adverse event whenever any employee becomes aware of an adverse event (see section [2.20](#) of this guidance for the specific employees FDA expects to be able to recognize that an adverse event requiring “remedial action” to prevent a risk of substantial harm to the public has occurred). Employees should be trained to properly identify, collect, and report complaints to your formally designated complaint handling unit [see 21 CFR [820.198\(a\)](#) for requirements related to complaint files].

User facilities and importers have specific mandatory reporting requirements [e.g., 21 CFR [803.10](#), [803.11](#), [803.20](#), [803.30](#), and [803.40](#)]. If a user facility notifies you of a death or serious injury and has not submitted a completed Form 3500A, we suggest that you remind the user facility of their obligation to do so within 10 work days after the day that it becomes aware of a reportable event. If the report involves a death, the user facility must also send the report to us.

If a user facility or importer uses the Form 3500A to report information to you about a reportable event, you can add your manufacturer information to that form and submit it to FDA (once you determine that you agree with the user facility that the event is reportable). If a user facility or importer does not use the Form 3500A to report information to you about an adverse event that is reportable under MDR, you need to provide information for all appropriate sections of the 3500A (or an electronic equivalent).

You may also receive adverse event information on a voluntary MedWatch Form 3500 or MedWatch Consumer Voluntary Reporting Form 3500B when there is no mandatory reporting obligation for the reporting entity (such as for user facilities reporting malfunctions or for physicians and consumers reporting deaths, serious injuries or malfunctions).

If the complaint information requires further investigation to determine if the event is MDR reportable, then your investigation should be conducted for the purpose of gathering enough evidence to support your decision to submit or not to submit an MDR report. This should be completed in a timely manner. Once you determine that the event is MDR reportable, you must submit the report to us within the required time frame [refer to 21 CFR [803.10](#), [803.11](#), [803.20](#), [803.50](#), and [803.52](#)]. You must submit the report to

us within 30 calendar days or 5 work days, whichever period of time is applicable [\[see, i.e., 21 CFR 803.10, 803.20, 803.50, and 803.53\]](#).

## **2.12 What are my responsibilities for investigating a voluntary report when the source of the report is unknown to me?**

You will need to evaluate the available information in the complaint to determine whether it represents an MDR reportable event [21 CFR [820.198\(a\)](#) and [803.18](#)]. Your evaluation must include information in your possession or that is reasonably available to you, such as information you can obtain by contacting a user facility, importer or other initial reporter related to the adverse event [[21 CFR 803.50](#)], if the source can be identified. Such information would include what is provided in the voluntary report, along with your knowledge of the device [21 CFR [803.18](#); see also 21 CFR [803.17](#), [803.50\(b\)](#), and [820.198](#)]. Your decision about whether or not the event is an MDR reportable event should be based on the findings of your evaluation.

## **2.13 What is a “serious injury”?**

An injury must meet the definition of “serious injury” in 21 CFR [803.3](#) for an event to be reportable as a serious injury. A “serious injury” is an injury or illness that [21 CFR [803.3](#)]:<sup>6</sup>

- Is life threatening;
- Results in permanent impairment of a body function or permanent damage to a body structure; or
- Necessitates medical or surgical intervention to preclude permanent impairment of a body function or permanent damage to a body structure.

“Permanent” means irreversible impairment or damage to a body structure or function, excluding trivial impairment or damage [21 CFR [803.3](#)]. Note that not all cosmetic damage will be considered trivial.<sup>7</sup> Furthermore, a life-threatening injury meets the definition of serious injury, regardless of whether the threat was “temporary.”<sup>8</sup>

It should also be noted that a device does not have to malfunction for it to cause or contribute to a serious injury. Even though a device may function properly, it can still

---

<sup>6</sup> One court agreed that “[a]lthough some of these consequences may be deemed clinically insignificant, they are considered to be serious injuries when coupled with the interventions, e.g. administration of antibiotics or other medications, explant, reconstruction, debridement, or revision surgery).” *TMJ Implants, Inc. v. U.S. Dep’t of Health & Hum. Servs.*, 584 F.3d 1290, 1301 (10th Cir. 2009).

<sup>7</sup> See the FDA response to comment 18 in Medical Devices; Medical User Facility and Manufacturer Reporting, Certification and Registration; Final Rule, [60 Fed. Reg. 63578, 63586 \(Dec. 11, 1995\)](#).

<sup>8</sup>See *id.* at 63587.

cause or contribute to a death or serious injury (see section [2.5](#) of this guidance for further discussion about caused or contributed).

## 2.14 What is a malfunction and when must a malfunction be reported?

“Malfunction” means the failure of a device to meet its performance specifications or otherwise perform as intended [21 CFR [803.3](#)]. Performance specifications include all claims made in the labeling for the device.

You do not need to assess the likelihood that a malfunction will recur. We presume that once the malfunction has occurred it will recur. In the preamble to the 1995 MDR final rule,<sup>9</sup> we stated that a malfunction is reportable if any one of the following is true:

- The chance of a death or serious injury occurring as a result of a recurrence of the malfunction is not remote;
- The consequences of the malfunction affect the device in a catastrophic manner that may lead to a death or serious injury;
- The malfunction results in the failure of the device to perform its essential function and compromises the device’s therapeutic, monitoring or diagnostic effectiveness, which could cause or contribute to a death or serious injury or other significant adverse device experiences required by regulation. (The essential function of a device refers not only to the device’s labeled use, but also to any use widely prescribed within the practice of medicine.);
- The malfunction involves:
  - a long-term implant or
  - a device that is considered to be life-supporting or life-sustaining and thus is essential to maintaining human life; or
- The manufacturer takes, or would be required to take, an action under section 518 or 519(g)<sup>10</sup> of the FD&C Act as a result of the malfunction of the device or other similar devices (see explanation of “similar device” below, and see section [2.21](#) of this guidance for an explanation of “remedial action”).

Although some may read the fourth criteria above as suggesting that anytime a malfunction of a long-term implant occurs it is per se reportable, that is not our intent. The malfunction of a long-term implant is reportable only when the malfunction would be likely to cause or contribute to a death or serious injury if it were to recur.

---

<sup>9</sup> See *id.* at 63585.

<sup>10</sup> The preamble reference is section 519(f), but the appropriate designation for the section is now section 519(g) due to amendments to the FD&C Act.

Malfunctions that are not likely to cause or contribute to a death or serious injury if they recur do not have to be reported.

We consider a malfunction that is or can be corrected during routine service or device maintenance to be an MDR reportable event if the recurrence of the malfunction is likely to cause or contribute to a death or serious injury. The fact that such a malfunction may be corrected does not change the fact that the device failed to meet its performance specifications or otherwise perform as intended.

We generally consider a device to be “similar” to another device if the devices have the same:

- Basic design and performance characteristics related to device safety and effectiveness; and
- Intended use and function; and
- Device classification and product code.

Devices that differ only in minor features unrelated to safety or effectiveness can be considered similar devices. Other factors that we recommend to be used to determine whether devices are similar include: brand name, common name, and whether the devices were introduced into commercial distribution under the same pre-market notification 510(k) (see 21 CFR Part 807) or the same pre-market approval (PMA) application number (see 21 CFR Part 814).

## **2.15 How do I decide whether a malfunction is “likely to” cause or contribute to a death or serious injury?**

Malfunctions that are reportable are those that reasonably suggest that one of the manufacturer’s marketed devices would be “likely to” cause or contribute to a death or serious injury if the malfunctions were to recur, even if the manufacturer may not have received information that the malfunction has caused or contributed to a death or serious injury. We describe certain malfunctions that we consider to be reportable in [Section 2.14](#) of this guidance.

For malfunctions other than those described in [Section 2.14](#) of this guidance, once the malfunction has caused or contributed to a death or serious injury, it is presumed that the malfunction is also “likely to” cause or contribute to a death or serious injury if it were to recur. Therefore, once a malfunction causes or contributes to a death or serious injury, you have an obligation to file MDRs for additional reports of that malfunction.

You should investigate and conduct follow-up to all additional complaints of such a malfunction to determine whether there have been any further deaths or serious injuries attributable to the same malfunction. A record of the investigation of the complaints

must be maintained as part of your MDR files in accordance with the MDR and QS regulations [21 CFR [803.18](#) and [820.198](#)].

An [MDR guidance for manufacturers](#) issued in 1997 stated that once a malfunction has caused or contributed to a death or serious injury, a presumption that the malfunction is likely to cause or contribute to a death or serious injury has been established. This presumption will continue until either the malfunction has caused or contributed to no further deaths or serious injuries for two years, or the manufacturer can show through valid data that the likelihood of another death or serious injury as a result of the malfunction is remote.

We continue to recommend that a firm develop valid data or information specific to the device and malfunction to support a conclusion that the malfunction is no longer likely to cause or contribute to a reportable death or serious injury if it were to recur.

FDA recommends that you submit a notification to FDA with a summary of the data and the rationale for your decision to cease reporting at the end of two years. FDA may, in certain circumstances, request additional information about the firm's decision within 30 calendar days of FDA's receipt of the notification. Your signed notification should be on your firm's letterhead and include the manufacturer report number of the last malfunction MDR you submitted to us. For mailing instructions, please refer to section 2.27 of this guidance.

Alternatively, you can request an exemption under 21 CFR 803.19(d) from further reporting sooner than two years if your analysis of the data supports your conclusion that the malfunction has not caused or contributed to further deaths or serious injuries and that the likelihood of another death or serious injury occurring as a result of the malfunction is remote. Your exemption request should include information such as the characteristics of the device, incidence rate for the malfunction, and assurance that complaints were investigated to support the rationale for the decision to cease reporting. For further information on exemption requests, refer to section 2.27 of this guidance. In this situation, your firm should continue to report until FDA has responded and granted the exemption request.

## **2.16 When does a complaint concerning a possible MDR reportable event not have to be reported?**

You must investigate all complaints of adverse events [21 CFR [803.17](#), [803.18](#), and [820.198](#)]. However, you are not required to submit an MDR report when:

- You have information that would enable a person who is qualified to make a medical judgment to reasonably conclude that your device did not cause or contribute to a death or serious injury, or that a malfunction would not be likely to cause or contribute to a death or serious injury, if it were to recur. Persons qualified to make a medical judgment include physicians, nurses, risk managers, and biomedical engineers. You must keep in your MDR event files the

information the qualified person used to make the reporting decision [[21 CFR 803.20\(c\)\(2\)](#)]. Your determination not to file an MDR should be supported by sufficient documentation to justify your decision. [21 CFR [820.198](#) and [803.18](#)]

- You determine that the information that you received is erroneous in that a device-related adverse event did not occur [21 CFR [803.22\(b\)\(1\)](#)].
- You determine that you did not manufacture the device [21 CFR [803.22\(b\)\(2\)](#)].
- You become aware of information from multiple sources regarding the same patient and the same event [21 CFR [803.22\(a\)](#)]. One report is required for the event when first reported and information from other sources may require the submission of a supplemental report to the initial MDR report.

You should retain documentation of erroneous complaints and your conclusion that an event is not an MDR reportable event in your MDR files for two years from the date of the event or a period equivalent to the expected life of the device, whichever is greater. The term “expected life” is explained in section [3.2](#) of this guidance [see 21 CFR [803.18\(c\)](#), [803.20\(c\)\(2\)](#), and [803.22\(b\)\(1\)](#)]. In addition, any reportable event information that is erroneously sent to you because it is a device made by another manufacturer must be sent to us with a cover letter explaining that you do not make the device in question [21 CFR [803.22\(b\)\(2\)](#)].

## **2.17 If I am a contract manufacturer, when do I need to obtain an exemption from reporting?**

The MDR regulation at [21 CFR 803.3](#) defines a “manufacturer” to include a firm that initiates specifications for devices that are manufactured by a second party for subsequent distribution by the person initiating the specifications. A contract manufacturer who does not distribute or market the devices it manufactures for a specifications developer would not have an MDR reporting obligation under [21 CFR 803.50](#) and would not require an exemption. However, if the contract manufacturer (Firm A) distributes or markets the devices that it manufactures for the specifications developer (Firm B), then both would have an MDR reporting obligation.

If Firm A and Firm B decide that they want only Firm A or Firm B to file the reports for the device, then the firm seeking the exemption must submit a request to us for an exemption from filing under 21 CFR [803.19\(b\)](#). We recommend that the two firms submit a joint request specifying which firm will submit the reports. Reporting exemptions are described in section [2.26](#) of this guidance. Instructions for submitting a request for exemption to us are in section [2.27](#) of this guidance. The request for exemption should provide a description of any agreement between the firms pertaining to MDR reporting for events involving the device. Both firms should maintain documentation of such agreement in their MDR procedures. Such an agreement does not negate a manufacturer’s responsibilities to comply with any exemptions, variances, or reporting alternatives that the FDA grants, or any other applicable requirement.

Information about the events, including any information about root cause and corrections, should be maintained by both firms. Additional conditions for this type of exemption will be specified in our written response to your request.

We may condition our approval of such an exemption on agreement by the firm requesting the exemption from reporting to be responsible for ensuring that the required reports are, in fact, submitted to FDA. If Firm A is designated to submit the MDR reports, but failed to do so, we would consider such a failure to report to be sufficient grounds to revoke Firm B's exemption from reporting. If revoked, both Firms A and B would be required to report MDR reportable events in compliance with all applicable MDR regulations.

## **2.18 What types of reports are required by the MDR regulation?**

You are required to submit three types of MDR reports. Each type of report must be submitted within the mandatory time frame. The following are the three types of reports:

1. 30-day (initial) reports [21 CFR [803.10\(c\)](#), [803.20](#) and [803.50](#)].
2. 5-day reports [21 CFR [803.10\(c\)](#), [803.20](#) and [803.53](#)].
3. Supplemental reports [21 CFR [803.10\(c\)](#) and [803.56](#)].

## **2.19 What are 30-day reports?**

A "30-day report" is the initial MDR report that must be submitted within 30 calendar days after the day you become aware of a reportable device-related death or serious injury, or a reportable malfunction [21 CFR [803.10\(c\)](#), [803.20](#) and [803.50](#)]. The report must be submitted as described in section [2.7](#) of this guidance. You will ordinarily submit 30-day reports for MDR reportable events, but you may be required to submit 5-day reports under certain circumstances as described in section [2.20](#) of this guidance.

Information that must be provided in your initial report includes:

- Patient information [21 CFR [803.52\(a\)](#)];
- Information about the adverse event or device problem [21 CFR [803.52\(b\)](#)];
- Device information [21 CFR [803.52\(c\)](#)];
- Initial reporter information [[21 CFR 803.52\(d\)](#)];
- Reporting information, including manufacturer contact information, and the report sources, date, type, and number [[21 CFR 803.52\(e\)](#)]; and
- Other information about the device and event [device manufacturer information, 21 CFR [803.52\(f\)](#)].

## 2.20 What are 5-day reports?

A “5-day report” (or five-day report) is a report that must be submitted to us within five work days after the day you become aware of an MDR reportable event:

- That necessitates remedial action to prevent an unreasonable risk of substantial harm to public health (remedial action is described in section [2.21](#) of this guidance); or
- For which we have made a written request for the submission of 5-day reports [21 CFR [803.10\(c\)](#), [803.20](#), [803.53](#)].

We consider the 5-day time frame for a report of an event requiring remedial action, as in the first bullet above, to begin the day after an employee with management or supervisory responsibilities over persons with regulatory, scientific, or technical responsibilities, or a person whose duties relate to the collection and reporting of adverse events, “becomes aware” of the event [21 CFR [803.3](#)]. We do not expect employees such as non-technical staff to recognize that an adverse event(s) requires remedial action to prevent a risk of substantial harm to the public.<sup>11</sup>

However, when you receive a written request for 5-day reports from us, you must submit the applicable reports within 5 work days after the day any employee becomes aware of the event [21 CFR [803.10\(c\)](#), [803.20](#), [803.53](#)].

## 2.21 What is a “remedial action,” and are all adverse events associated with “remedial actions” reportable to FDA as 5-day reports?

Under the MDR regulation, a “remedial action” is any action, other than routine maintenance or servicing of a device, necessary to prevent recurrence of an MDR reportable event [21 CFR [803.3](#)]. FDA does not consider an action taken to correct only a single device involved in an MDR reportable event to be a remedial action.

A decision that a remedial action is necessary can be based on any information, including the occurrence of one or more MDR reportable events, or internal analyses, such as trend analyses, using appropriate statistical or other acceptable methodologies [21 CFR [803.53\(a\)](#)]. Not all MDR reportable events requiring remedial actions need to be submitted as 5-day reports. Only events that require remedial actions to prevent an unreasonable risk of substantial harm to the public health or events for which FDA requests such a report must be submitted as 5-day reports [21 CFR [803.53](#)]. When the remedial action taken is not required to address an unreasonable risk of substantial harm to the public health [21 CFR [803.50](#)] the reportable adverse events should be submitted as 30-day reports instead of 5-day reports.

---

<sup>11</sup> See [60 Fed. Reg. at 63581-63582](#).